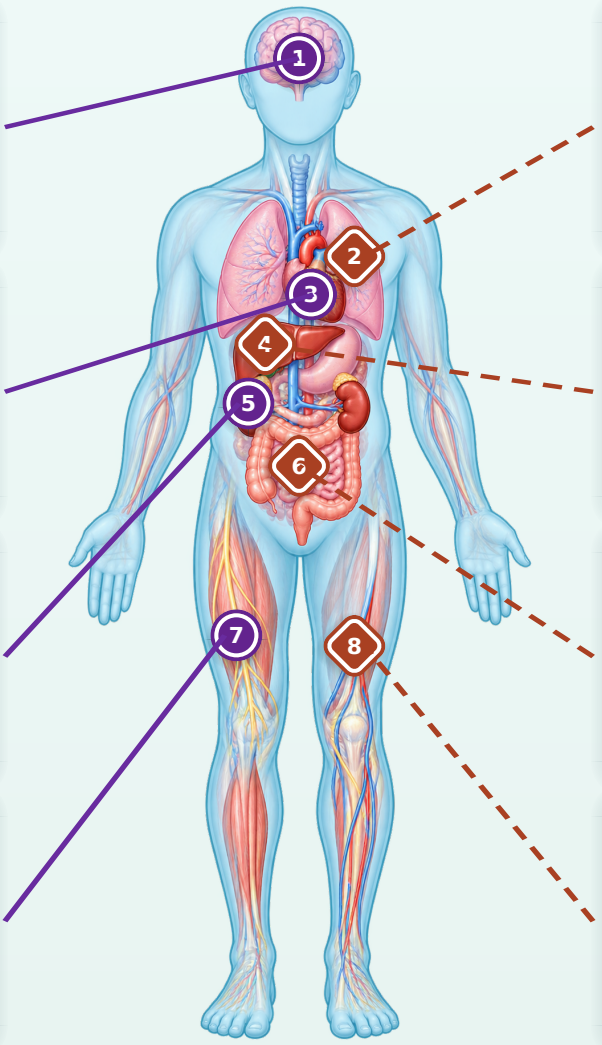
Nivolumab, ipilimumab, paclitaxel and carboplatin

Cycle 1 lasts 42 days: nivolumab, ipilimumab, paclitaxel and carboplatin on Day 1; nivolumab, paclitaxel and carboplatin on Day 22. These are the only two planned chemotherapy administrations. From Cycle 2, nivolumab is given on Days 1 and 22 and ipilimumab on Day 1. Continue while appropriate until progression, unacceptable toxicity or up to 24 months without progression. Your team reviews the plan and may delay or stop treatment.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Hormones / brain

Nivolumab / ipilimumab

Unusual weakness, headache, dizziness, thirst or passing much more urine.

2 Lungs

Nivolumab / ipilimumab

New cough or breathlessness needs prompt assessment; infection or cancer can cause similar symptoms.

3 Heart / muscles

Nivolumab / ipilimumab

Chest pain, racing heartbeat, fainting or new muscle weakness needs urgent assessment.

4 Liver

Nivolumab / ipilimumab

Yellow eyes/skin, dark urine or abnormal tests can signal inflammation.

5 Kidneys

Carboplatin / immune medicines

Kidney tests matter; report less urine, swelling or difficulty drinking.

6 Bowel / mouth

Paclitaxel / immune medicines

Report new diarrhoea, pain, blood/mucus or mouth ulcers; do not assume ordinary chemotherapy upset.

7 Skin / nerves / muscles

Paclitaxel / immune medicines

Report spreading rash, blistering, numbness, weakness or difficulty swallowing.

8 Blood / infection

Paclitaxel / carboplatin

Fever, shivers, sudden illness or unusual bleeding need immediate oncology advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Follow your oncology team’s emergency instructions for fever, shivering or sudden illness, including with normal temperature. Report new breathlessness/cough, diarrhoea or abdominal pain, reduced urine, severe rash, unusual weakness, headache or marked thirst promptly. Severe chest pain, collapse, confusion or major breathing difficulty needs emergency care.

NCCP 00712 · PRACTICAL SUPPORT

Your treatment, at home.

Important medicine-specific effects

Nivolumab

- Immune inflammation in multiple organs
- Infusion reactions
- Delayed effects after treatment stops

Ipilimumab

- Immune bowel, liver, skin and hormone inflammation
- Other serious organ inflammation
- Infusion reactions

Paclitaxel

- Low blood counts and infection
- Peripheral neuropathy and hair loss
- Allergic reactions
- Muscle and joint pain

Carboplatin

- Low blood counts and infection
- Nausea and vomiting
- Kidney, hearing and nerve effects
- Allergic reactions

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Cycle 1 Days 1 and 22 · chemotherapy-containing visits

Cycle 1 lasts 42 days: nivolumab, ipilimumab, paclitaxel and carboplatin on Day 1; nivolumab, paclitaxel and carboplatin on Day 22. These are the only two planned chemotherapy administrations. From Cycle 2, nivolumab is given on Days 1 and 22 and ipilimumab on Day 1. Paclitaxel premedication includes corticosteroid and antihistamine treatment, with H2-blocker decisions made by the team. The source allows consultant-directed changes; ask for a written plan for each of the two chemotherapy visits. The team must reconcile paclitaxel allergy-prevention steroids with anti-sickness steroids and the aprepitant interaction into one prescription. Do not add courses or take leftover steroids yourself. Use dedicated carboplatin AUC≥4 NCIS row; no routine olanzapine in that row. Myalgia/arthralgia may need an individual analgesia plan. Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.

Cycle 2 onwards · Day 1 nivolumab + ipilimumab

Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose. Chemotherapy is complete; review and stop induction prophylaxis rather than automatically repeating it.

Cycle 2 onwards · Day 22 nivolumab alone

Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00712/

Sources and review status are available online. Follow your own prescription and hospital instructions.